

Public Health Emergency Operations Center

"COUSP DRC"

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°068/MVB_21/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (47)	- Haut-Uélé (4/13): Wamba, Boma Mangbetu, Pawa and Isiro - Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa - Tshopo (3/23): Makiso-Kisangani, Lubunga and Mangobo
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Publication date	July 22, 2026

INDICATORS KEYS — CURRENT TO JULY 21, 2026

CONFIRMED CASES — 5 PROVINCES 2,536 +63 new cases (Ituri 46 · N-Kivu 12 – Haut-Uele 5)	CONFIRMED DEATHS · LEATHITIS 1,033 · (40.7%) +34 deaths (Ituri 26 · North Kivu 6 – Upper Kivu Uele 2)	SUSPECTED CASES OF THE DAY 306 Ituri 215 · N-Kivu 88 · Tshopo 3
PATIENTS IN ISOLATION 738 Ituri 556 · North Kivu 182	CURED — CUMULATIVE 506 +24 recovered (Ituri 23 – North Kivu 1)	NATIONAL CONTACT TRACKING 77.2% 10,765/ 13,936 views · Ituri 75.4% · N-Kivu 81.7% · Tshopo 100.0%

1. Epidemiological Highlights

- **Epidemiological situation** : No new affected health zones have been reported. The total remains at 47 affected health zones out of 140 in the five provinces, of which 46 remain in active transmission.
- **Coordination** : Installation and start-up of MVE diagnostic analyses in the province of Haut-Uele; Workshop for the development of the MVE 17 information management guide.
- **Case trends** : In the last 24 hours, 63 new confirmed cases, 34 confirmed deaths and 24 recoveries were recorded:
 - o **Ituri**: 46 new confirmed cases were notified, mainly in Bunia (24), Rwampara (9), Drodoro (3), Fataki (3), Boga (2), Nia-Nia (2), Lita (1), Mandima (1) and Nizi (1) and 26 deaths among which 38.5% were community deaths.
 - o **North Kivu**: 12 new confirmed cases were reported (Katwa 6, Butembo 4 and Beni 2) as well as 6 deaths, including 83.3% community transmission and 16.7% at the CTE. The provincial total reaches 259 confirmed cases and 152 deaths, representing a case fatality rate of 58.7%.

- o **Haut-Uélé** : 5 new confirmed cases and 2 community deaths have been reported in the health zone of Isiro. The provincial total rises to 21 confirmed cases and 12 deaths, representing a case fatality rate of 57.1%.
- o **South Kivu and Tshopo** : No new confirmed cases or deaths have been reported. The totals remain at 5 cases, 4 deaths, and 1 recovery in Tshopo, and 3 cases, including 1 death, in South Kivu, where the Miti-Murhesa health zone has a total of 55 days without a new confirmed case.
- **Analysis:** Ituri remains the epicenter of the epidemic, accounting for 88.6% of cumulative cases and 73.0% of new daily confirmations. Contact tracing performance remains insufficient and requires strengthened surveillance activities.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 47 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.

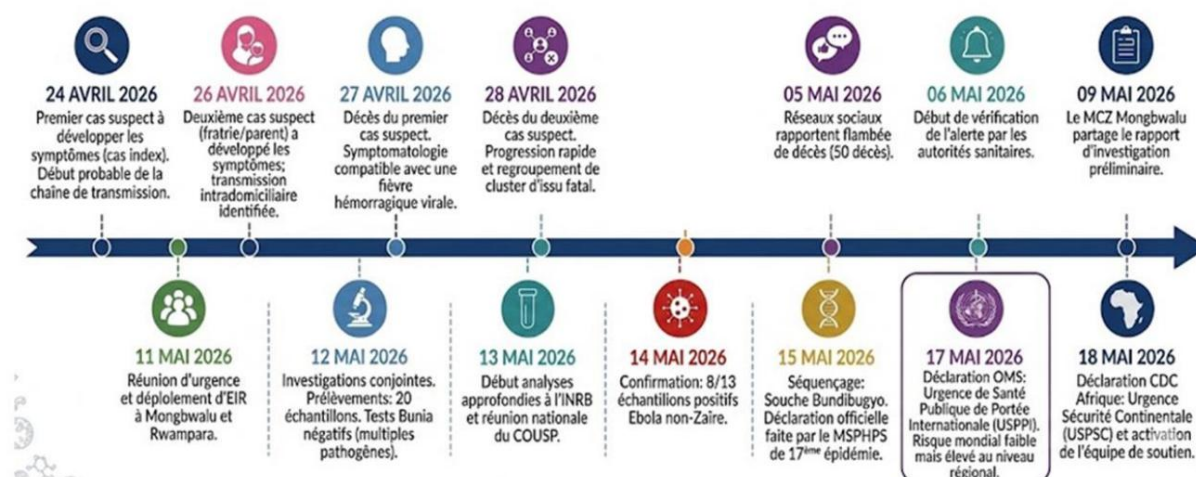


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	2,248	864	38.4%	28 / 36 (77.8%)	46
North Kivu	259	152	58.7%	11/34 (32.4%)	12
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Haut-Uélé	21	12	57.1%	4/13 (30.8%)	5
Tshopo	5	4	80.0%	3/23 (13.0%)	0
Total	2,531	1033	40.7%	47 out of 140	63

The Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 88.8% of confirmed cases (n = 2,228) and 83.8% of deaths (n = 864), representing a case fatality rate of 38.4%. The health zones of Bunia (681 cases), Rwampara (466),

Mongbwalu (364), Nizi (224) and Nyankunde (99) remain the most affected and concentrate approximately 81.6% of cases reported at the provincial level and 72.5% at the national level.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	2,248	864	38.4%	46	10	16	26
Bunia	681	228	33.5%	24	4	5	9
Rwampara	466	150	32.2%	9	2	8	10
Mongbwalu	364	206	56.6%	0	0	2	2
Nizi	224	99	44.2%	1	0	0	0
Nyankunde	99	28	28.3%				
Lita	79	28	35.4%	1	1	0	1
Mangala	61	38	62.3%				
Nia-Nia	56	17	30.4%	2	0	1	1
Bamboo	40	14	35.0%				
Komanda	27	11	40.7%				
Tchomia	23	7	30.4%				
Unidentified	17	0	0.0%				
Fataki	14	7	50.0%	3	0	0	0
Kilo	11	3	27.3%				
Adja	10	1	10.0%				
Mandima	11	6	54.5%	1	1	0	1
Mambasa	8	6	75.0%				
Logo	8	1	12.5%				
Ariwara	10	1	10.0%				
Damascus	7	0	0.0%				
Aungba	6	2	33.3%				
Drodro	8	4	50.0%	3	1	0	1
Rimba	5	0	0.0%				
Aru	4	2	50.0%				
Kambala	2	2	100.0%				
Lolwa	2	1	50.0%				
Boga	3	2	66.7%	2	1	0	1
Gety	1	0	0.0%				
Mahagi	1	0	0.0%				
North Kivu	259	152	58.7%	12	5	1	6
Katwa	114	78	68.4%	6	4	0	4
Butembo	65	31	47.7%	4	0	0	0
Blessed	39	27	69.2%	2	1	1	2
Musienene	19	5	26.3%				
Kyondo	10	5	50.0%				
Oicha	4	4	100.0%				
Kalunguta	2	1	50.0%				
Masereka	3	0	0.0%				
Goma	1	0	0.0%				

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Vuhovi	1	1	100.0%				
Mabalako	1	0	0.0%				
South Kivu	3	1	33.3%	—			—
Miti-Murhesa	3	1	33.3%				
Haut-Uélé	21	12	57.1%	5	2	0	2
Wamba	6	3	50.0%				
Pawa	3	3	100.0%				
Boma Mangbetu	4	2	50.0%				
Isiro	8	4	50.0%	5	2	0	2
Tshopo	5	4	80.0%	—			—
Makiso-Kisangani	3	3	100.0%				
Mangobo	1	1	100.0%				
Lubunga	1	0	0.0%				
Total	2,536	1033	40.7% 63		17	17	34

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwatu, Nizi and Nyankunde, with an extension towards North Kivu (Katwa, Butembo, Beni), Haut-Uélé (Isiro and Wamba) and Tshopo.

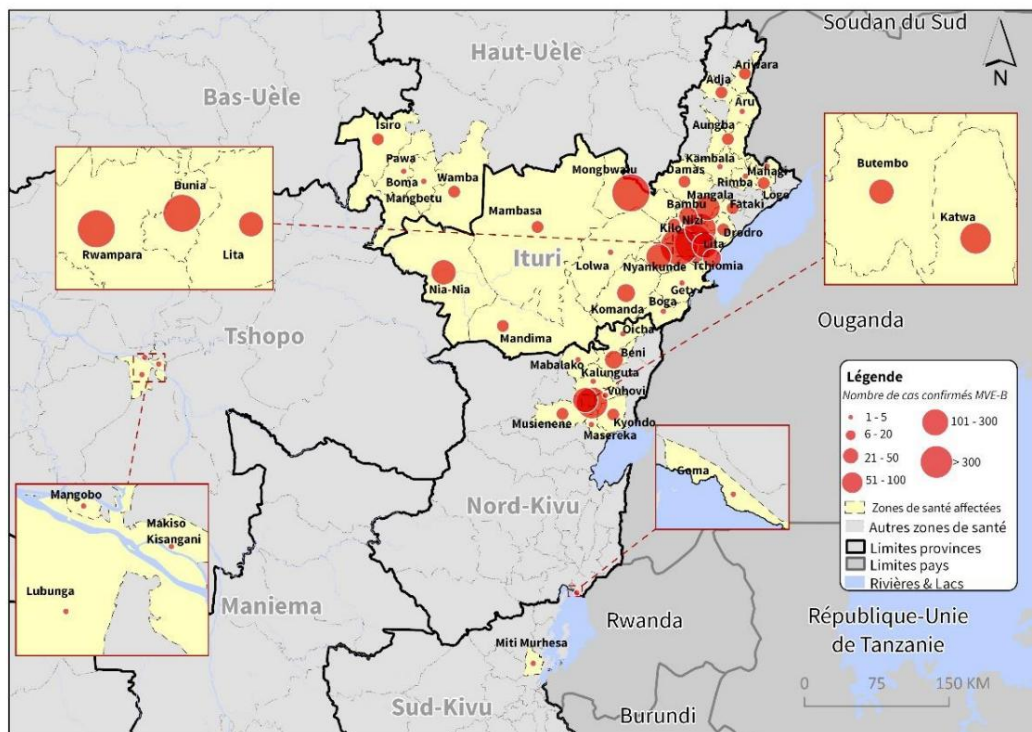


FIGURE 2 — HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES (AS OF 21/07/2026)

The recent emergence of new outbreaks in Haut-Uélé confirms a progressive expansion of the affected areas, highlighting the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S25 to S30, with fluctuations related to notification delays and the gradual consolidation of data.

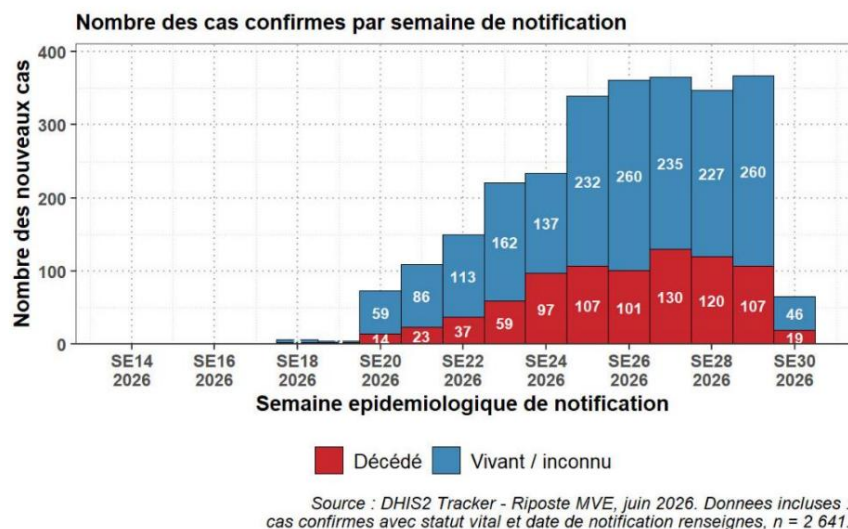


FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

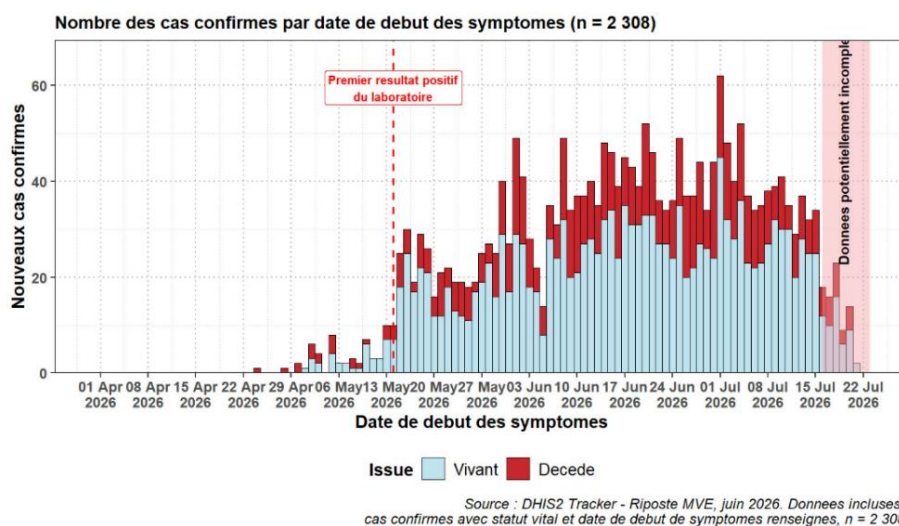


FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

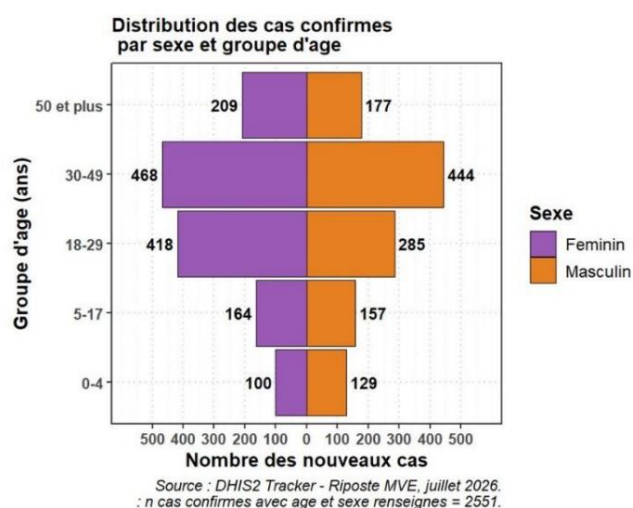
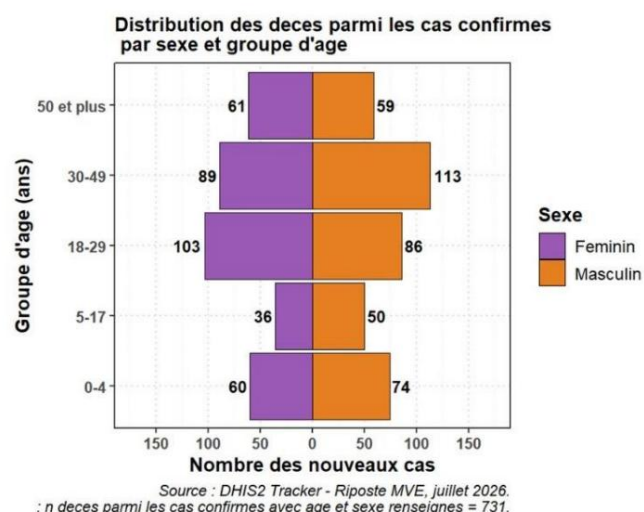


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS AS OF 20/07/2026

Indicator	Ituri	North Kivu	Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	13	2	ND	1	ND	16
New alerts received	485	911	ND	3	ND	1399
Total alerts for the day	498	913	ND	4	ND	1415
Alerts investigated	332	902	ND	3	ND	1237
Investigation rate (24 h)	66.7%	98.8%	ND	75.0%	ND	87.4%
Alerts confirmed — live	160	55	ND	3	ND	218
Alerts confirmed — deceased (comm.)	55	33	ND	0	ND	88
Total suspected cases for today	215	88	ND	3	ND	306

In total, 1,415 alerts (1,399 of which were new) were received, with 1,237 investigated within 24 hours (87.4%) and 306 validated (218 active cases and 88 community deaths). This improvement reflects a reduction in investigation delays and the consolidation of data from the affected provinces. The improved investigation rate is linked to the reduction in investigation delays and the strengthening of surveillance teams in the affected provinces.

3.2 Laboratory: tests and positivity

ŷ **Ituri** : 132 samples documented in the collection network, of which 46 were positive (36 living and 10 deceased), i.e., a positivity rate apparent of 34.8%.

ŷ **North Kivu** : 122 samples received and analyzed, of which 12 were positive (7 living and 5 deceased), representing a positivity rate of 9.8%.

ŷ **Haut-Uele** : 20 samples received and analyzed, of which 5 were positive (3 living and 2 deceased), representing a positivity rate of 25.0%.

- Installation and start-up of MVE diagnostic analyses in the province of Haut-Uele.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts being followed	Viewed contacts	tracking rate
Ituri	10,100	7,616	75.4%
North Kivu	3,761	3,074	81.7%
Tshopo	75	75	100.0%
Haut-Uélé	ND	ND	ND
South Kivu (end of monitoring)	0	0	N / A
Total	13,936	10,765	77.2%

Contact tracing remains insufficient at the national level (77.2%), below the operational threshold of 95%. Ituri only managed to follow up with 7,616 out of 10,100 contacts (75.4%). North Kivu achieved a performance of 3,074 out of 3,761 contacts (81.7%). Tshopo maintains a high performance with a 100% follow-up rate. Follow-up is now complete in South Kivu.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 10 alerts notified

Six (6) alerts were notified to the PoE/PoC in Ituri, including four live alerts and two intercepted bodies. Of these, 4 live alerts were validated and all referred to the CTE/CT.

North Kivu — 0 alerts notified

No alerts were notified to the various points of entry (PoE) and points of control (PoC).

South Kivu — ND

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC), AS OF 21/07/2026

Indicators	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (15/15)	100% (75/75)
Number of people who used PoE/PoC % of travelers	103643	698 24	173467
screened % of travelers who	93%	98%	95.5%
washed their hands	92%	98%	95%
% of travelers made aware	97.4%	98%	97.7%
Number of alerts notified	6	0	6
Number of validated live alerts	4	0	4
Number of bodies recovered today	2	0	2

Indicators	Ituri	North Kivu	Overall
Number of problematic contacts intercepted today	0	0	0
Percentage of validated live alerts referred to the CT/CTE	66.7%	N / A	66.7%
% of lifeless bodies swabed	N / A	N / A	N / A
Number of positive results of referred suspected cases	0	0	0

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI), AS OF 21/07/2026

Indicator	Ituri	North Kivu	Haut-Uele	Tshopo	Together	
Number of beds	708	141	ND	ND	849	
Patients in bed (Day -1)	556	176	ND	ND	732	
Total admissions (24 hours)	68	24	ND	ND	92	
Total admissions	654	200	ND	ND	854	
Exits	Deceased	22	1	ND	ND	23
	No case	20	15	ND	ND	35
	Healed	23	1	ND	ND	24
	Escapees	3	1	ND	ND	4
	Transferred to the HGR	0	0	ND	ND	0
	Total outflows (24 h)	68	18	ND	ND	86
Patients in isolation (end of day),	556	182	ND	ND	738	
including confirmed cases	257	17	ND	ND	274	
including suspects	299	165	ND	ND	464	
Occupancy rate	78.5%	129.1%	ND	ND	86.9%	

In total, 738 patients were in isolation, including 274 confirmed cases and 464 suspected cases, for an overall occupancy rate of 86.9 % (738 patients out of 849 beds), with 854 admissions and 86 discharges (24 recovered, 23 deceased, 35 non-cases, and 4 who escaped). Ituri bears the brunt of the burden with 556 patients in isolation (257 confirmed, 299 suspected) out of 708 beds, representing an occupancy rate of 78.5%. In North Kivu, 182 patients are in isolation (17 confirmed, 165 suspected) for an occupancy rate of 129.1% (141 beds). Data from Tshopo and Haut-Uele provinces were unavailable at the time of compilation.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY INDICATORS SMSPS, JULY 20, 2026

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	13 (72.2%)	1 (100%)
Confirmed cases are being monitored.	126 (95.5%)	2 (100%)
New suspected cases	25 (55.6%)	3 (100%)
Suspected cases are being monitored	82 (59.9%)	13 (100%)
Laboratory results announced (including positive ones)	46	1
Children separated — nursery / community	10	1
CTE support staff	316	6
Frontline Personnel (PPL)	30	2

SMSPS Key Indicators	Ituri	North Kivu
Members of households and communities	39	0
EDS supported (SMSPS)	9	0

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD (CUMULATIVE AS OF JULY 21, 2026, ITURI)

Health Zone	Confirmed cases	Cured.	Hospitalized.	Death
Bunia	35	18	3	14
Rwampara	32	24	1	7
Mongbwalu	16	6	2	8
Nyankunde	14	11	1	2
Nia-Nia	10	3	7	0
Lita	4	1	2	1
Nizi	3	0	0	3
Mangala	3	1	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Total — case fatality rate 29.8%	121	67	18	36

PPL = healthcare personnel and frontline providers.

7 Pillar-Based Response Activities

COORDINATION ÿ Ituri : Holding of coordination meetings; installation from the Logo laboratory; monitoring of response activities; deployment of CAP investigators; infrastructure monitoring. ÿ Tshopo : Finalization/validation of the response plan; establishment of the DPS alert center; field recommendations for CTE sites.	PCI / WASH ÿ Ituri: 8 rings opened; 29 exposed sites identified and decontaminated; 17 assessments; 40 ESS monitored; 7 ESS and 6 means of transport decontaminated; 82 PPL trained; 22/110 households decontaminated. ÿ North Kivu: Detailed PCI/WASH data not visible in the presentation received on 21/07. ÿ Tshopo : 3 ESSs supplied with inputs; CS evaluation Boyoma (16%) and CTE HCKIS (82%); PK23 RN4 supplied with 500 L of water
Dignified and secure burials (DSM) ÿ Ituri: Official handover of 6 EDS vehicles to Mongbwalu; start of EDS training in Ngamau; provision of 35 EDS kits to Nizi; resistance in Bambu and Rwampara; continuation of decontamination base. ÿ North Kivu: 38 death alerts, 37 bodies swabbed/secured, 1 failed swab in Butembo, 34 EDS carried out, 12 postponed; 89.4% of deaths in ESS. ÿ Tshopo: Coordination required for the management of community deaths and the safe transport of bodies.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿ Ituri: continued supervision of the Health Zone.

COMMUNICATION (RCCE / CREC) ŷ Ituri: 2,759 people were reached by awareness messages delivered by RECO 476 in households in 2 of the 4 Health Zones (NIANIA, BUNIA); 3,424 people were reached during mass awareness sessions held by community actors in Health Zones (BUNIA, NIANIA); 4,253 people were reached with awareness themes during the Awareness-raising sessions held by community actors in the Health Zone. (BUNIA, NIANIA; Civil society advocacy in Bunia; advocacy by Bandaka/Bombo leaders in Nia-Nia; 2,759 people reached through direct communication; 1,657 through discussions/dialogues; 3,424 through mass communication; 44 community alerts; 42 feedback/infodemics) ŷ North Kivu: Visit of journalists to the CTE-Beni; briefing of military/police; dialogue with hotel owners in Beni; broadcast on Radio IMARA; 17,843 people reached door-to-door; 3,158 in public places; 209 community alerts. ŷ Tshopo: Briefing for journalists, taxi drivers, community leaders and the provincial CREC; 360 people reached via direct mail; 15,868 in mass awareness campaigns; training underway for 100 community leaders in Mangobo	HOLISTIC CARE (HCC) ŷ Ituri: 24 new recoveries including 23 in Ituri and 1 in North Kivu, 70 admissions, 556 patients in isolation, 16 deaths among discharges including 68 confirmed, 980 hot meals served to patients including 120 to suspected cases, 130 to confirmed cases, 691 to PPL and 39 to accompanying persons. ŷ North Kivu: 24 admissions, 18 discharges, 175 patients hospitalized, including 2 confirmed and 22 suspected; continued monitoring of care in CTE/CT. ŷ Tshopo: 9 hot meals were served to the patient, including 6 to suspected cases and 3 to confirmed cases.
RESEARCH, INNOVATION & VACCINATION ŷ Ituri: Publication of the results of the anthropological study in the Mongbwalu health zone. ŷ Tshopo: Mini-KAP survey recommended to guide community feedback	LOGISTICS ŷ Ituri: Continuation of support by pillar. ŷ North Kivu : Reception of nutritional supplies; packaging and delivery of PEC supplies to Beni; deployment, mitigation, and security in Beni; awareness-raising for T2 agents and detainees; screening of 65 people/visitors at PPEVS ŷ Tshopo: Received 7 tablets; delivered passenger information sheets to PoE Bangboka and PCI kits; 3 vehicles and 1 ambulance assigned; visited/geolocated CTE large capacity/EOC site.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ŷ Ituri: Briefings for psychologists and IMC staff; PEAS training for hygiene CTE Rwampara, Awareness-raising on protection against EAS at the CTE of ISTM NYANKUNDE in BUNIA, 10 psychology agents at CTEM: 4 women and 6 men as well as 6 family members of patients; Briefing for 18 men and 14 women psychologists from the CTE of HGR Bunia and 10 health structures in the Bunia health zone; 11 new IMC staff briefed on safeguarding and AAP including 10 men and 1 woman.	

8 Main challenges, impacts and actions required

Challenge No.	Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of governor military of Ituri)	Interruption of response activities, including dignified and safe burials Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT: occupancy of 129.1% in North Kivu (bed capacity: 141) and 78.5% in Ituri (556 patients for 708 beds);	Admission delays, increased risk of nosocomial transmission Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity.
4	Low contact tracing performance in Ituri (77.2%); North-Kivu 81.7%	Uninterrupted transmission chains Train, deploy and motivate community liaison officers (RECOs); strengthen supervision; investigate the observed drop in Ituri
5	Insufficient number of ambulances and pre-positioning at PoC	Delays in transfer and isolation Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Weakening of overall care Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (175/1170, i.e. 15%)	pupil infections Nosocomial risks (121 cases of PPL including 36 deaths) Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of laboratories proximity (Butembo, Mambasa and Logo)	Long confirmation delays, transmission not detected Accelerate the deployment of decentralized diagnostic capabilities
10	Financial shortcomings of the resources	Limitation of the implementation of the pillars Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



Official handover of 6 vehicles for EDS activities in the Mongbwalu health zone



Journalists' visit to the CTE Beni



Deployment of inputs in the Beni health zone

**POUR TOUTE
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Avec l'appui technique de l'Organisation Mondiale de la Santé (OMS),
US CDC, Africa CDC et Blue square